

Patient ID: SallenPeriod: February 6 to March 7, 2024Report Date: March 8, 2024Coverage: 502/735h (54.3%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Feb 06 to Mar 07

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
23	24h	Very High Breathing	<1	68%

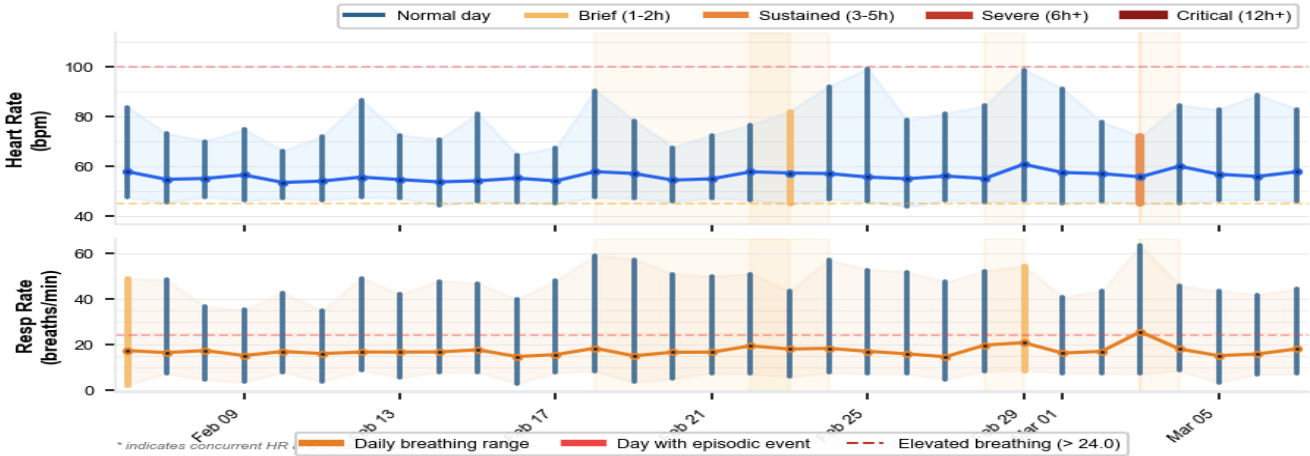
Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Very High Breathing	63 brpm	2h	1h	2	26 brpm	Mar 03
2	High Breathing	43 brpm	2h	1h	1	19 brpm	Feb 22 to Feb 23
3	High Breathing	45 brpm	1h	1h	1	26 brpm	Mar 03
4	Elevated Breathing	54 brpm	6h	2h	2	20 brpm	Feb 28 to Feb 29
5	Elevated Breathing	57 brpm	8h	1h	1	17 brpm	Feb 18 to Feb 24
6	Elevated Breathing	43 brpm	2h	1h	1	22 brpm	Mar 03 to Mar 04

RR values outside physiologic range (6–60 breaths/min).

Clinical Pattern Observations:

- 1. Breathing variability: RR spread 10 brpm (12 to 23).
- 2. Clustered pattern: Episodic events on 4 of 31 days (12%).



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
Very High HR > 110 bpm
- Low HR < 45 bpm
Elevated Breathing > 24 brpm
- Elevated HR > 95 bpm
High Breathing > 30 brpm
- High HR > 100 bpm
Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.